

INITIAL MANAGEMENT OF IGA VASCULITIS (HENOCH SCHÖNLEIN PURPURA; HSP)

1. BACKGROUND & POINTS FOR COUNSELLING

- IgA vasculitis (IgAV) affects children 3 to 15 years old; peak incidence ~ 5 years of age.
- Approximately 50% preceded by a URTI (especially Streptococcal infection)
- Without renal disease = good prognosis, typically resolves within 4 weeks.
 - No recurrence in 2/3 of patients
 - Recurs in 1/3 of patients (within 4 months of initial presentation)
- Joint pain resolves within 72H; Abdominal pain resolves within 24 – 48H.
- Renal involvement = long term complications
 - Occurs in 20 to 50% of patients.
 - Onset may be delayed (90% within 2 months; 97% within 6 months)
- Steroid therapy does not prevent renal involvement.

2. CLINICAL FEATURES

Diagnostic Criteria for IgA Vasculitis¹ = **Purpura** & at least **one of the following**:

- Abdominal pain,
- Arthritis/arthritis,
- Renal involvement

CRITERION	GLOSSARY	
Purpura (mandatory)	Symmetrical distribution over extensor surfaces – typically over lower limbs, buttocks; can also affect face (behind ears) and upper limbs. Spares the trunk.	
Abdominal pain	Diffuse colicky abdominal pain with acute onset.	
Arthritis or arthralgia	Joint pains typically involve large joints of the lower limbs.	
Renal involvement ²	Proteinuria	UPCR >0.02 g/mmol (> 2 years) >0.05 g/mmol (< 2 years)
	Hematuria	UFEME > 2 RBC/hpf

¹ EULAR/PRINTO/PRES 2010 classification criteria of IgA vasculitis.

² Adapted from UK Kidney Association Clinical Practice Guidelines for the Initial Management of IgA vasculitis (Henoch Shonlein Purpura) in Children & Young People (Dec 2022)

2.1 SYSTEMIC COMPLICATIONS ASSOCIATED WITH IGAV

SYSTEM	DEFINITIONS
GI	Intussusception, GI bleeding, pancreatitis, protein losing enteropathy
Urology	Scrotal edema, orchitis, testicular torsion
Pulmonary	Pulmonary hemorrhage
CNS	Cerebral vasculitis, seizures, encephalopathy
Renal	Microscopic hematuria, proteinuria, hypertension, nephritic/nephrotic syndromes, RPGN ³ & renal failure

3. MANAGEMENT IN CE

3.1 SCREENING FOR RENAL INVOLVEMENT

- Urinalysis (dipstick)
 - If any hematuria ($\geq 1+$), → follow up with UFEME
 - If proteinuria ($\geq 1+$), → follow with up spot UPCR
- Hypertension (BP > 95th centile)
- Patients with proteinuria $\pm$ hematuria, or hypertension should have a baseline renal panel performed.
- Patients with proteinuria, hypertension or abnormal renal function should be admitted.

Test	Positive Result
UFEME	> 2 RBC/hpf
UPCR	>0.02 g/mmol (> 2 years) >0.05 g/mmol (< 2 years)

3.1.1 Isolated Microscopic Hematuria

- Dipstick positive for only Hb with normal renal function and blood pressure.
- Holds a good prognosis and generally does not require treatment.
- Order UFEME for confirmation.
- Patients can be discharged with early clinic review after urine sample collected (no need to wait for results). See "[Discharge](#)".

3.2 SURGICAL EMERGENCIES

- Obtain pediatric surgical consult for cases of:
 - Suspected intussusception
 - Testicular pain / painful scrotal purpura / suspected testicular torsion
- Imaging: Ultrasound study for evaluation of intussusception & testicular torsion
- Start corticosteroids for patients with severe abdominal pain/GI bleed/orchitis.

³ RPGN – rapidly progressive glomerulonephritis

3.3 SYMPTOM MANAGEMENT

SYMPTOM / COMPLICATION	DRUG			
	Drug	Dose	(max.)	Freq.
Joint pains	PO Paracetamol	15mg/kg	(1 g)	q6H
Non-specific abdominal pain	PO Ibuprofen	10mg/kg	(400 mg)	q8H
	PO Naproxen sodium	11mg/kg	(550 mg)	q12H
Painful scrotal purpura / orchitis / testicular torsion	PO Prednisolone	0.5 – 1mg/kg	(60 mg)	q12H
Intussusception / severe abdominal pain / GI bleeding	IV Hydrocortisone	4mg/kg	(100 mg)	q6H

4. DIFFERENTIAL DIAGNOSIS

- In atypical presentations, to consider investigations if clinically suspicious for:
 - Hematological malignancy
 - Autoimmune disease (e.g., SLE, ANCA-associated vasculitis)
 - Meningococemia (irritability, purpuric rash in a non-verbal infant)
 - Bleeding diathesis (coagulopathy/ITP)
- Suggested investigations:
 - FBC, PT/PTT/INR
 - LFT, LDH
 - CRP, ESR, blood culture
 - ANA, SLE panel (C3/4, anti-dsDNA)
 - Anti-Neutrophil Cytoplasmic Ab (ANCA)
 - IgA

5. INDICATIONS FOR ADMISSION

PRESENTATION	DETAILS
Severe GI complications	Intussusception, GI bleeding, severe GI pain refractory to oral analgesia, associated with vomiting or affecting oral intake
Severe MSK complications	Arthritis/arthritis with significant limitation in function (affects activities of daily living) refractory to oral analgesia
Renal involvement	Hypertension, proteinuria, macroscopic hematuria, abnormal renal function
Urologic complications	Orchitis, scrotal edema/purpura
Pulmonary & CNS complications	Pulmonary hemorrhage Cerebral vasculitis (encephalopathy, seizures, neurological deficits)

6. DISCHARGE

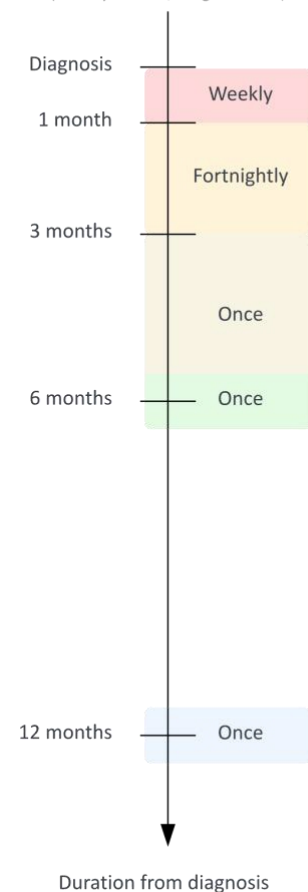
	DISCHARGE PLANS	
☐ Medications	Analgesia <i>Prednisolone (only if indicated)</i> ○ Maximum duration of 2 weeks (to encourage compliance to follow up) ○ Dose requires tapering over 2 to 4 weeks	
☐ Return advice	Severe abdominal pain, bloody stools, bilious vomiting Gross hematuria, altered mental state, seizures, breathlessness	
☐ Follow up	No renal involvement	General Paediatrics x 1 week (comments: "force book as per HSP guidelines")
	Isolated microscopic hematuria	1st line: Paeds Renal x 1 week 2nd line: General Paediatrics x 1 week <i>Patients with persistent isolated microscopic hematuria at 1 month should be referred to Paeds Renal.</i>

6.1 OUTPATIENT CLINIC REVIEWS

Counselling caregivers

- Frequent reviews to be expected in the first 6 months following diagnosis.
- This is because 90% of patients with renal involvement present by 2 months of diagnosis; and 97% by 6 months of diagnosis.
- The diagram on the right shows an example that can be used to counsel parents re: frequency of follow up.

Frequency of Outpatient Reviews
(Urinalysis & BP/weight checks)



7. REFERENCES

Reid-Adam, J. (2014). Henoch-Schönlein Purpura. *Pediatrics In Review*, 447-449.

(2022). *UK Kidney Association Clinical Practice Guidelines for the Initial Management of IgA vasculitis (Henoch Schönlein Purpura) in children and young people*. UK Kidney Association.